

19 May 2023

Lucy Letby - Cross-examination Day 3: Babies C & D

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MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Friday, 19 May 2023

(10.30 am)

(In the absence of the jury)

MR JUSTICE GOSS: Yes, Mr Myers.

MR MYERS: May it please you, my Lord, there's one matter I'd be grateful to raise before we continue today. It won't take long. It's just a concern, so I raise it now in this way before we proceed, coming out of one aspect of the cross-examination of Ms Letby yesterday relating to [Baby C].

For your Lordship's note and anyone's reference, it's the passage in the transcript between pages 129 to 135. It was relating to Ms Letby's assessment of the condition of [Baby C] and it began with the question:

"Do you accept that [Baby C] progressed well having been born in good condition?"

And the aim, the purpose of it, which was dealt with part-way through, was the question, "It all paints parents a positive picture", to which she assented.

That question was not based just upon matters relating to her experience, but more generally. We appreciate the task faced by the prosecution, as it is by us, in fact, in traversing the evidence in this case in a practicable way. But what happened over about four pages of transcript and a couple of minutes of court time was a fairly swift survey of the component parts of a week of evidence, much of which Ms Letby had no particular involvement with, in fact all of it, concluding with a swift assessment or statement, rather, of the view of Dr Marnerides that there was no bowel obstruction, simply put like that. It was said:

"You'll remember he said there was no bowel obstruction or words to that effect. The histology did not show he had any bowel issue?"

And Ms Letby said yes and that was that.

That is a matter that is hotly contested and on which there is a lot of evidence competing. Our concern is that it is one thing for her involvement to form the basis of questioning like that. It certainly isn't, with respect, we submit, appropriate for her to engage really in what becomes the provision of an expert opinion, which is what happens once we get into the territory of inviting her to comment on expert evidence, or putting it to her, putting it before the jury, of course, and one part of the expert evidence before the jury, as a statement.

I have raised the matter with my learned friend Mr Johnson King's Counsel and he will respond as he sees fit, but we would say it is not an approach that we have taken with other witnesses and certainly we have not embarked upon rehearsing pieces of expert evidence with non-experts for them to comment upon as if they are experts or expect them to do so.

So our principal concern is treating Ms Letby as, in effect, a surrogate expert on what are key issues of expert evidence, ultimately, which draw upon a careful analysis of what has been sometimes days, if not a week, of evidence.

There is a subsidiary issue, we respectfully observe, to be borne in mind: given her circumstances as a witness in this case, it is a task and a half for her to absorb a mass of detail at speed, putting up different charts or nursing notes on the system that she then has to look at and is asked questions like, "Do you agree with what..." or, "Just have a read of the rest of that, what do you think", or picking out one side of the IC cares chart that has breathing support but not assisting her by identifying the repeated references to bile aspirates that run through it.

It's a matter for Ms Letby to read carefully what is put in front of her so far as she can. That is for her.

But our concern is she is assisted with relevant details so far as possible. I say that recognising, as I'm bound to from what I have been dealing with, the task Mr Johnson is dealing with.

So I don't say this as any type of slight or criticism, but as a matter of caution, to bear in mind the defendant's position, but over and above that we would ask that she not be invited to, in effect, become a surrogate expert on matters that go beyond what she or any other nurse has been expected to deal with in the case. We know there are general matters of nursing practice that she will be asked about but when we get down to questions of, for example, bowel issues from the evidence of Dr Marnerides, it's important, we submit, that she not be drawn into that and it's for that reason that we just raise that as carefully and respectfully as we can, my Lord.

MR JUSTICE GOSS: Thank you, Mr Myers.

Mr Johnson.

MR JOHNSON: There are a number of issues there, my Lord.

The first is the speed of the cross-examination. I have repeatedly said to the witness that if she needs more time to read a document then of course she shall have it.

MR JUSTICE GOSS: And you will continue to do that. If I think you are forcing something at too quick a pace, I will intervene. So I will be alert to the point that Mr Myers is making and if I think you are in any way being inappropriate or too quick or too selective in relation to matters, I will intervene.

MR JOHNSON: Yes. The second is selection. My learned friend, unsurprisingly, in chief was very selective --

MR JUSTICE GOSS: Well, I'm not overly impressed because the -- I understand the point that Mr Myers is making, but throughout this case there have been, as there always are -- one side is particularly selective in relation to certain pieces of evidence and perhaps passes over other pieces of evidence. But if you -- what I will simply say, Mr Myers, is if you consider, in relation to any particular line of questioning, it is unfairly selective and presents an inaccurate picture of the clinical situation at that time, then you should raise it. If I think it does, I will, but otherwise -- which is no more than common practice. We'll do it that way.

Finally, the surrogate expert witness point.

MR JOHNSON: I don't accept that as a criticism at all, my Lord. An example of the way it was articulated to me was that it was not acceptable to be referring the witness to notes or opinions of other people and asking her to comment.

There are two issues here. The first is that my learned friend repeatedly, and I am not complaining about this, but repeatedly set the scene in each case.

And we have a record of the times between when my learned friend started talking in each case and the time that he first asked questions.

MR JUSTICE GOSS: I know. I think what I will say in relation to this issue is that you are certainly not precluded from drawing the witness's attention, the defendant's attention, to unchallenged expert evidence conclusions --

MR JOHNSON: Yes.

MR JUSTICE GOSS: -- and then asking questions about that.

You certainly are not precluded from doing that. But if it is a controversial expert opinion then it may be more difficult. Obviously, she can't give clinical evidence.

She has, as is evident from her evidence, a wealth of experience despite her relatively short period of nursing and a lot of knowledge about medical matters, but you'll know where the boundary is and if I think you're going over the boundary, Mr Johnson, I will intervene. Otherwise, we'll leave that. All right, Mr Myers?

MR MYERS: I'm grateful, my Lord, thank you.

MR JUSTICE GOSS: I hope that's clarified the ground rules.

MR MYERS: Yes, thank you.

MR JUSTICE GOSS: Good. Thank you very much. The usual situation: choose an appropriate moment after an hour or so.

MR JOHNSON: Certainly.

MR JUSTICE GOSS: Thank you very much.

Jury in, please.

(In the presence of the jury)

MR JUSTICE GOSS: Good morning, members of the jury. I'm sorry we are late coming in, I was just dealing with a matter. We're in a position to proceed now. Sorry about that delay.

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JOHNSON: Yesterday at the end of the day, we'd got to the position in the chronology or the sequence of events relating to [Baby C] just before he collapsed at 23.15; do you remember that?

LETBY: Yes.

JOHNSON: I had just been asking you about a part of your interview; do you remember that?

LETBY: Yes.

JOHNSON: And I was asking you, as well as the part of your interview, about some text messages that you had sent to your friend, Jennifer Jones-Key.

LETBY: Yes.

JOHNSON: Just to put us back into the sequence, the text messages were sent at between 23.01 and 23.09. They're between tiles 174 and 180. Okay?

LETBY: Yes.

JOHNSON: I was asking you questions specifically about what I was suggesting to you was your acceptance that at the time, this is in interview, your acceptance in interview that at the time [Baby C] collapsed, you were in nursery 1. Do you remember that?

LETBY: Yes.

JOHNSON: And do you accept that you were in nursery 1 at the time [Baby C] collapsed?

LETBY: No.

JOHNSON: Why did you tell the police that you did accept it?

LETBY: Based on what Sophie Ellis saw and what was told to me, I took that to be accurate. I have no memory of being in the room.

JOHNSON: If you have no memory, how can you dispute?

LETBY: I dispute the fact that I have not agreed that I was definitely in the nursery.

JOHNSON: I'll put the question a different way then. Are you disputing that you were in the room at the time [Baby C] collapsed?

LETBY: Yes, I have no recollection of that.

JOHNSON: Well, I'm going to suggest to you -- I'm not going to do this every time because otherwise we'll be here a long time, but I'm going to suggest to you that you're ignoring the question and in due course I may suggest to the jury that the reason you're ignoring the question is because you can't answer the question. All right? So I'll give you one more opportunity to answer it.

Do you dispute that you were in the room at the time [Baby C] collapsed?

LETBY: Yes.

JOHNSON: Why?

LETBY: Because I have no memory of that.

JOHNSON: Do you remember being born?

LETBY: No.

JOHNSON: Do you dispute that you were born?

LETBY: No.

JOHNSON: Just before the prosecution say that you were in the room with [Baby C] when he collapsed, you were texting Jennifer Jones-Key.

LETBY: Yes.

JOHNSON: You had left one of your babies in the nursery, having just taken his or her observations; do you remember that?

LETBY: I can't say when I left that baby, no. I don't know where I did the texting.

JOHNSON: Well, let's just look at this if we can, please. Can you go to the neonatal review at page 3 of 9? Can you see there at line 65 the fact that you took the observations of the baby PE?

LETBY: Yes.

JOHNSON: And PE was in nursery 3, wasn't she?

LETBY: Yes.

JOHNSON: Do you remember PE as a matter of interest?

LETBY: No.

JOHNSON: Can you see what's in the line above that?

LETBY: Yes, Nicola Dennison has made an entry on the fluid balance chart.

JOHNSON: Yes. Just to remind us, who is Nicola Dennison?

LETBY: The band 4 nursery nurse.

JOHNSON: Why did you leave PE with Nicola Dennison?

LETBY: I haven't handed over care to Nicola Dennison.

JOHNSON: No, but whether or not you've handed over care, why did you leave a nursery nurse, a band 4, someone of lesser qualification to you, with a nursery nurse (sic)?

LETBY: Well, I can't answer that. It's not unusual for nursery nurses to help out with feeds.

JOHNSON: Well, it may not be, but I'm asking why you left that child at that time with that nursery nurse.

LETBY: I have no memory of that.

JOHNSON: Whose responsibility was PE?

LETBY: Mine.

JOHNSON: Did you have something better to do?

LETBY: No. Both babies that night had the care they needed.

JOHNSON: That's not the question. I'm not suggesting that Nicola Dennison couldn't do what you left her to do.

What I'm asking you is why you left her to do it.

LETBY: I can't recall that now.

JOHNSON: Well, I will remind you. Can we go to tile 174? To moan to Jennifer Jones-Key is the answer, isn't it?

LETBY: No.

JOHNSON: Sorry?

LETBY: No, I've not left somebody to go to text. I've also been doing things with JE as well at that time.

JOHNSON: Oh, show us on the neonatal review, please, what you were doing with JE.

LETBY: 55 and 56.

JOHNSON: Where's that?

LETBY: Lines 55 and 56.

JOHNSON: Right. Yes. Okay. But we know that that time is the wrong time, don't we?

LETBY: It's not a wrong time, it's an approximation.

JOHNSON: Sorry, could we go back to the text, please? According to your evidence that you gave to the jury, you never use your phone in a clinical area.

LETBY: No, that's right.

JOHNSON: So you were not in the nursery at 23.01, were you?

LETBY: No.

JOHNSON: If you hadn't been texting your mate, would you have had time to feed JE -- sorry, PE?

LETBY: I did feed JE at that same time, which is possibly why Nurse Dennison looked after PE for me.

JOHNSON: Would you answer the question, please? If you hadn't been texting your mate, would you have had time to feed PE?

LETBY: I can't answer that.

JOHNSON: Because you were texting Jennifer Jones-Key right up to 23.09, weren't you?

LETBY: Yes.

JOHNSON: Eight minutes. That might well have been long enough to feed your baby, mightn't it --

LETBY: Yes.

JOHNSON: -- PE?

LETBY: I can't recall the circumstances around that now.

JOHNSON: But it took you out of the nursery, didn't it, whatever the circumstances were?

LETBY: Yes, I would not have used my phone at the cot side, so yes, I would have been in the doorway or elsewhere on the unit, yes.

JOHNSON: This is the time we suggest that you were in nursery 1 with [Baby C]; yes?

LETBY: I don't accept that, no.

JOHNSON: Mel Taylor wasn't in the room when [Baby C] collapsed, was she?

LETBY: I don't recall her evidence.

JOHNSON: Well, that's her evidence and if we look at tile 185, please:

"Called to help baby. When arrived to baby, baby apnoeic."

She told us she wasn't in the nursery, didn't she?

LETBY: I can't remember her saying that, but she is saying she's come over to the baby, yes, to the baby's cot side. She doesn't specify being out of the nursery.

JOHNSON: That was her evidence.

LETBY: Okay. I can't recall her evidence right here, right now.

JOHNSON: Do you remember how Mel described you --

LETBY: No.

JOHNSON: -- at this time? "Cool and calm"; do you dispute that?

LETBY: No, I would expect members of staff to be cool and calm on a unit. That's what we're trained to do.

JOHNSON: And you said to Sophie Ellis:

"He's just had a brady and a desat."

LETBY: I can't recall saying that.

JOHNSON: Do you dispute that you said it?

LETBY: I have no memory of saying it.

JOHNSON: Well, I'll ask you one more time: do you dispute that you said it?

LETBY: Yes, because I have no memory of that.

JOHNSON: Do you remember the arrival of Dr Katherine Davis at 23.26?

LETBY: No, not specifically. I remember doctors arriving but I couldn't be specific on times, no.

JOHNSON: Can we go to tile 187, please. This is Dr Davis' -- I think this is the wrong tile, actually, for the note.

Sorry, no, it isn't, sorry. My fault:

"23.26, crash bleeped to attend NICU. On arrival, Guedel airway in situ, bagged via Neopuff by nursing staff. Occasional intermittent gasps noted. Chest compressions commenced."

Can we scroll down, please?

MR JUSTICE GOSS: I think actually, going back up -- sorry to be pedantic about this, but I am a pedant -- it says:

"Bagged via Neopuff by nursing staff. Chest compressions in progress on arrival."

MR JOHNSON: Sorry, yes.

MR JUSTICE GOSS: And then "chest compressions recommenced" three lines further down.

MR JOHNSON: Yes, quite right. I was just trying to -- if Mr Myers thinks I'm missing anything out, I'm sure he will --

MR JUSTICE GOSS: It's not Mr Myers, it's me. I'm just reading what we can all see on the note.

MR JOHNSON: Yes. We can see the condition of [Baby C] there:

"Occasional intermittent gasps."

Is that right?

LETBY: Yes.

JOHNSON: Is that what you saw, for example, with [Baby I] on 13 -- I can't remember which month it was now. 13/10, wasn't it, 2015?

LETBY: I'm not sure that's that date, is it, for [Baby I]?

JOHNSON: I think it was the second incident, 13 October. Anyway. You know the incident I'm talking about in nursery 2?

LETBY: Yes.

JOHNSON: The same sort of thing, is it?

LETBY: Yes.

JOHNSON: And then if we can scroll down, please. We have the two attempts at intubation, but:

"Unable to pass ETT as cords ++. Clear oral secretions, no blood seen. Neopuffed with Guedel airway."

Cords ++. She told us, Dr Davies, that they were swollen and oedematous, was the word she used. What does oedematous mean?

LETBY: Swollen.

JOHNSON: Had you put something down [Baby C]'s throat?

LETBY: No.

JOHNSON: Do you agree that this becomes a theme of some of these collapses, that when the doctors look down the throats of these children, they see either swelling or bleeding?

LETBY: Yes, I know other doctors have commented on that, yes.

JOHNSON: Yes. This is not something that's caused by the use of a Guedel airway, is it?

LETBY: I can't answer that.

JOHNSON: There then ensued a very long and protracted decline for [Baby C], didn't there?

LETBY: Yes.

JOHNSON: And [Baby C] had massive gastric dilation and ballooning of the stomach, didn't he?

LETBY: At this point?

JOHNSON: Well, that's the evidence of what was found after the event.

LETBY: Okay.

JOHNSON: Yesterday I was asking you about Sophie Ellis having aspirated [Baby C]'s stomach just before she fed him that one half of 1ml of milk --

LETBY: Yes.

JOHNSON: -- at 11 o'clock. Do you agree that something caused [Baby C]'s stomach and bowel to dilate between 11 o'clock and his collapse?

LETBY: So the air you're referring to is before the collapse?

JOHNSON: That's what's found after he died. We're suggesting that's the reason for the collapse.

LETBY: I think if he was found with air when he's died, that could be related to the Neopuff resuscitation. I'm not accepting that he had noticeable gas between 23.00 and 23.15.

JOHNSON: So something caused him to -- he was fed at 23.00?

LETBY: Yes.

JOHNSON: Something caused him to collapse at 23.15?

LETBY: Yes.

JOHNSON: Extensive CPR was undertaken?

LETBY: Yes.

JOHNSON: For some reason, a heart rate was restored; do you remember that?

LETBY: Yes.

JOHNSON: And he clung to life for a further 5 hours plus?

LETBY: Yes.

JOHNSON: Had you ever seen that sort of a decline before?

LETBY: In the context of when he first collapsed?

JOHNSON: Yes.

LETBY: Yes, I've seen babies suddenly collapse before, yes.

JOHNSON: And cling to life the way [Baby C] did?

LETBY: No, I haven't seen that before, no.

JOHNSON: You enjoyed the aftermath of this, didn't you?

LETBY: No.

JOHNSON: Why were you so keen to involve yourself with the [Family of Baby C] as they cradled their dying son?

LETBY: I don't agree with that. I don't recall being there for a lot of the time like they've said.

JOHNSON: Do you remember [Nurse B] said that she had to keep pulling you out of the family room?

LETBY: Yes.

JOHNSON: That was not disputed on your behalf with her, was it?

LETBY: I can't recall that now.

JOHNSON: Do you dispute it now?

LETBY: That I'd been in the family room?

JOHNSON: Repeatedly.

LETBY: No, I agree that I've probably gone round at some point but not repeatedly, no. I don't recall [Nurse B] having to pull me back out.

JOHNSON: What she said was this:

"I asked Lucy to focus back in nursery 3 on baby JE because I was still heavily concerned about him. However, Lucy went into the family room a few times and I asked her to come out and to leave that family [that is the [Family of Baby C]] with Mel."

Are you disputing that?

LETBY: Yes.

JOHNSON: Of course, Mel is senior to you, isn't she?

LETBY: Yes.

JOHNSON: More experienced?

LETBY: Yes.

JOHNSON: Given that Mel was looking after this family, what useful function were you performing?

LETBY: I -- I don't understand the question, sorry.

JOHNSON: What were you contributing --

LETBY: I can't recall from --

JOHNSON: -- to the dreadful situation that the [Family of Baby C] were going through?

LETBY: I can't recall whether I assisted Mel with any of the mementos, which is a two-person job.

JOHNSON: You can't recall it, so what, are you disputing that you did assist her?

LETBY: No, I agree that I went to the family room at times.

It's the conversation with [Nurse B] that I don't recall.

JOHNSON: Right. So given that you are agreeing with what she says, that you did go in there several times, I will return only once more to the question: what useful contribution were you making for this family?

LETBY: I can't recall from my memory now.

JOHNSON: So does that mean that you dispute that you were making a useful contribution?

LETBY: I'd have to check the charts to see if there was anything that I co-signed at that time.

JOHNSON: The charts wouldn't be in the family room, would they?

LETBY: The bereavement checklist, yes.

JOHNSON: You leave that with the family, do you, the bereavement checklist?

LETBY: All the paperwork for a baby is kept in their cot side, so --

JOHNSON: He wasn't in a cot, he was in the arms of his mother --

LETBY: No, but I may have been assisting --

JOHNSON: -- dying, wasn't he? Wasn't he, Lucy Letby?

LETBY: Yes.

JOHNSON: Yes. So one more time: tell us what you were doing.

LETBY: I can't recall from memory. I would have to see if there was something that I signed as doing with Mel, such as the hand and footprints.

JOHNSON: Of course if that was what you were doing, [Nurse B] would have approved of it, wouldn't she?

LETBY: I have no memory of that conversation with [Nurse B].

MR JUSTICE GOSS: Are hand and footprints taken when the baby is still alive?

LETBY: Yes. Either like that or when they've passed.

MR JOHNSON: Do you remember what else [Mother of Baby C] told the jury about what you did?

LETBY: I'd made a comment about saying goodbye to [Baby C] and putting him in a basket.

JOHNSON: Yes:

"You've said your goodbyes now, do you want to put him in here?"

Agreed evidence. Do you still agree it or do you dispute it?

LETBY: No, I don't recall ever saying that, no.

JOHNSON: So you're disputing it now, are you?

LETBY: Yes.

JOHNSON: Even though it's agreed evidence. I see.

When did you decide that you were disputing that bit of evidence?

LETBY: I don't understand.

JOHNSON: It was agreed when it was read to the jury, you see, so that's the starting point. We're now at the end point where you're disputing it. I'm asking you when between when it was read and today you decided that you were disputing it.

LETBY: I don't think I've ever accepted definitively that that's what I said, if that's an accurate recollection by the parents, but I don't recall that.

JOHNSON: You were enjoying what was going on, weren't you, Lucy Letby?

LETBY: No.

JOHNSON: Did you remember, before you saw all the evidence in this case, that [Baby C]'s collapse had happened just after he was first given that one half of 1ml of milk?

LETBY: Yes.

JOHNSON: Why did you remember that particular detail?

LETBY: Because Sophie Ellis had been very upset that [Baby C] had deteriorated so soon after having his first feed that she had given him.

JOHNSON: Was she blaming herself?

LETBY: She was questioning whether it was related, yes. She was very upset.

JOHNSON: But you knew what had really happened, didn't you?

LETBY: No.

JOHNSON: Did you enjoy seeing Sophie Ellis upset?

LETBY: No.

JOHNSON: I'm going to move on to the case of [Baby D].

I want to start, if we may, with your defence statement, please, so if you can find that typed document in front of you.

It starts, this section of the document, at paragraph 58. Have you got that there?

LETBY: Yes.

JOHNSON: I'd be grateful if you would read out paragraphs 58 to 66 as these are your words.

LETBY: "I did nothing to hurt [Baby D]. I was not in the nursery before 19.30 on 21 June 2015. When I did come on duty I was not looking after [Baby D] and I would not have been with her and able to have a conversation there with her mother. I did not have a conversation with [Mother of Baby D] about bubbles around [Baby D]'s mouth."

JOHNSON: I'll take over because there's an issue and I don't want there to be any problems.

At 60 you say:

"I think the nurse described as being at [Baby D]'s cot at 19.00 to 19.20 is Kate Bissell. It would have been this nurse that [Mother of Baby D] spoke to and not me."

61:

"I have no specific recollection of the collapse at 01.30. I was probably in nursery 1 when this happened."

62:

"I don't think I had any involvement with [Baby D] until the resuscitation. She was not my allocated baby."

63:

"I think I was with Caroline Oakley at 03.00 or 03.45 when the collapse occurred. I can't remember who found [Baby D] or what state she was in."

I say for the benefit of the jury, my Lord, that they will get this in writing in due course. I'm sorry it's not yet been provided. It might save them making a note.

MR JUSTICE GOSS: I'm sorry, yes, I should have said.

Mr Johnson did say you would get it. It's got names in it that have to be redacted and things like that, so don't worry about getting a note, you will get this document because you're obviously -- the witness has been asked questions about this document and you'll have it.

MR JOHNSON: "I did not do the footprints or the hand prints for [Baby D]."

Okay?

LETBY: Okay.

JOHNSON: Were you affected at all by the death of [Baby D]?

LETBY: We were all affected on the unit that night, yes.

JOHNSON: Would you answer for yourself rather than including other people? Were you affected by it?

LETBY: Yes.

JOHNSON: You didn't, for example, think something like, well, it's happened and that's it?

LETBY: No.

JOHNSON: You've said in evidence, and indeed to the police, that you can't remember much about [Baby D], haven't you?

LETBY: That's right.

JOHNSON: And that is not true, is it?

LETBY: No, I believe that is true.

JOHNSON: Let's see what the evidence shows. Can we start, please, Mr Murphy, with tile 168 of the sequence?

If we could just look at the children and the staff, please, rather than -- thank you.

So we see, don't we there, that nursery 1 was where [Baby D] was?

LETBY: Yes.

JOHNSON: Caroline Oakley was her designated nurse?

LETBY: Yes.

JOHNSON: ME was also in there. ME, as a matter of interest, was the same ME that you had been looking after the night [Baby C] died.

LETBY: JE, do you mean?

JOHNSON: Well, JE and ME actually.

LETBY: I think it was PE.

JOHNSON: You're quite right.

LETBY: I don't think that's the same baby.

JOHNSON: No, no, I'm wrong, you're right, PE -- sorry, JE.

LETBY: Yes, I recall JE.

JOHNSON: Yes.

LETBY: And then MRE is a different baby.

JOHNSON: Yes. Thank you.

So they were in nursery 1, you were looking after them?

LETBY: Yes.

JOHNSON: Three babies in nursery 2 with Caroline Oakley responsible for one of them?

LETBY: Yes.

JOHNSON: And therefore Caroline Oakley was splitting her time between nursery 1 and nursery 2?

LETBY: Yes.

JOHNSON: And it follows, doesn't it, that therefore you would have been alone in nursery 1 from time to time?

LETBY: At times, yes.

JOHNSON: I'm going to start by reminding you, if we could take that down, Mr Murphy, thank you, of some evidence that [Mother of Baby D] gave to the jury on 3 November last year.

LETBY: Okay.

JOHNSON: [Mother of Baby D], of course, is [Baby D]'s mum. This is in relation to the resuscitation:

"I couldn't really see [Baby D]. The way he [and that's a reference to Dr Brunton] was holding her, I was seeing more of his back and we were standing there looking at [Baby D] dying. Somebody was holding a phone to his ear and he kept shouting, 'This is [Baby D]', and he was quite agitated. Then Dr Newby told him that he had to let [Baby D] go, he had to stop, and she insisted and she tapped on his shoulder and said, 'You've got to let her go'. So he held her but he stopped massaging her and he pronounced the time of death. I couldn't stay in the room anymore so then I asked [Father of Baby D] to take me away."

And [Father of Baby D] is [Baby D]'s dad. Okay?

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: Sorry?

LETBY: Yes. I remember her saying that, yes.

JOHNSON: You were the nurse holding the phone to Dr Brunton's ear, weren't you?

LETBY: No, I don't agree with that.

JOHNSON: Oh. [Mother of Baby D] identified you as that nurse, didn't she, in her statement?

LETBY: Yes, but I can't confirm that that was me.

JOHNSON: Right. So you dispute it because you can't remember it even though it's agreed evidence?

LETBY: I'm saying I have no memory of doing it.

JOHNSON: Do you dispute it?

LETBY: I don't think I understand.

JOHNSON: Do you understand what the word "dispute" means? It's your word I'm using, you see. You tell us what you mean by dispute as you've used it.

LETBY: That if we're going to accept what Sophie said as true then that's different, but me remembering it, I dispute that that actually happened from my memory. I would be reliant on her accounts being accurate but I can't confirm that either way.

MR MYERS: That's a lot like saying "simply not accepting", if that's the case, which is a position we've had before. That's all I'm pointing out. We've had before witnesses saying they don't remember and then they are asked, well, are you saying it didn't happen or you can't remember. We're being caught repeatedly on a question of whether not accepting is disputing. I'm just making that observation, that's all.

MR JOHNSON: Yes. There's a difference between agreed evidence, of course, because you are given the opportunity to go through these statements before they're read to the jury, aren't you?

LETBY: Yes.

JOHNSON: Yes -- sorry, quite right. [Mother of Baby D] gave evidence, I'm so sorry, my mistake. [Mother of Baby D] actually said this in court --

LETBY: Yes.

JOHNSON: -- and it wasn't disputed on your behalf.

LETBY: Okay.

MR JUSTICE GOSS: Well --

LETBY: I can't recall that.

MR JUSTICE GOSS: Do you remember a phone being held by someone to the doctor's ear?

LETBY: Yes, I do recall that the incident happened, yes.

MR JUSTICE GOSS: So you can remember there was such an incident.

LETBY: Yes, because it was talked about after the event.

MR JOHNSON: Because the wrong mother was phoned, wasn't she?

LETBY: Yes.

JOHNSON: That's the problem.

LETBY: I agree it happened, but I cannot say that it was me that made that phone call.

JOHNSON: I want to move on to other bits of evidence and see whether you dispute these. Kathryn Percival-Calderbank, she was in nurseries 3 and 4; do you remember that or not?

LETBY: Yes.

JOHNSON: What is her experience as you remember it?

LETBY: She's a very experienced senior nurse.

JOHNSON: Been working as a nurse for more than 30 years, hasn't she -- hadn't she?

LETBY: Yes.

JOHNSON: She said this:

"It was on her body, her arms and her -- so all from her chest downwards, her trunk and her arms, and it wasn't like a normal rash that you would know if a baby was becoming septic because they tend to be -- the blood vessels seem to be more bluey, they're quite mottled. This was more -- there seemed to be a larger like mosaic-type rash and it was a reddy-blue. The reddy-brown colour in it was more like the vessels or like oval-type markings on the skin, but like the vessels of blood or whatever it was seemed to be meeting up on each other."

So that is evidence she gave on 9 November. It's at pages 62 to 64 of the transcript if anybody wants to check.

Was she a friend of yours?

LETBY: Kate Percival-Ward?

JOHNSON: Yes.

LETBY: I wouldn't say we were close, but we got on well, yes.

JOHNSON: Is she mistaken about what she saw?

LETBY: Well, again, if that's what she saw, I can't comment on what she did or didn't see.

JOHNSON: Do you dispute it?

LETBY: I did not see that.

JOHNSON: Well, I'm not going to go round the houses with you again.

Caroline Oakley --

MR MYERS: My Lord, it's not a question of going round the houses. The witness has said she didn't see that.

We're back to where we were yesterday with descriptions being given. There may be issues for reliability and credibility for the jury to draw their conclusions on, but that's a comment made voicing my learned friend's opinion. We'll save our opinions for the conclusion of this case. But she simply said, "I didn't see it", and that's as far as that can go. It's not something to be commented on in that way when she's trying to remember the question in that way, we submit, but it --

MR JOHNSON: "Round the houses" is asking, "Well, do you dispute it?" I'm not going to go round that house again because we've done it so many times.

MR JUSTICE GOSS: As I indicated yesterday, you are perfectly entitled to put forward the description of another witness and ask the defendant: did you see that or any of it or anything like that?

MR JOHNSON: Yes.

MR JUSTICE GOSS: And she says, "No, I didn't".

LETBY: It's the term "dispute" I'm a little bit unsure of because I can't comment on what anybody else saw; I just know what I did or didn't see.

MR JOHNSON: Well, you were there, weren't you?

LETBY: I was not there for the whole event, no.

JOHNSON: We'll come to that. We'll carry on with these for now.

Caroline Oakley, who we've established was in nurseries 1 and 2:

"I don't remember specifically the exact rash, but I remember it was -- I hadn't seen it before. It was dark, it was unusual, and the rash struck me. I hadn't seen that rash before on a baby I've looked after. I've put discolouration. I remember it as like a deep red/brown, different than mottling, different from what I have seen before."

If anyone wants to check that, 4 November last year, page 45.

Was Caroline Oakley a friend of yours?

LETBY: Not outside of work, no.

JOHNSON: Is she a very experienced nurse?

LETBY: Yes.

JOHNSON: Is she mistaken in what she saw?

LETBY: No, that sounds similar, I agree with that.

JOHNSON: [Nurse C], who we saw from the population chart was in nursery 2.

LETBY: Yes.

JOHNSON: "I remember her skin was discoloured and that she was quite stiff in appearance, like a mottled appearance, which is like white circles of skin with a discolouration around it, which was like a reddy colour. It was an odd -- I remember it because it was an odd rash. I would say commonly in a baby that might need some attention is -- the baby will be blue or grey and the mottling, that colour mottling is around the white patches, whereas this was not that colour.

I remember from my statement this morning that I've written that it was like a reddy-brown colour. It was unusual because it didn't match a colour that I had seen before. It was like circles with a blood supply around them."

Again, if anyone wants to check, that's 8 November last year, pages 65 to 67.

Was Caroline Oakley a friend of yours?

LETBY: No.

JOHNSON: Sorry, [Nurse C]. I beg your pardon.

LETBY: No.

JOHNSON: Did you have issues with [Nurse C]?

LETBY: No.

JOHNSON: Is she a very experienced nurse?

LETBY: Yes.

JOHNSON: Approximately how much experience?

LETBY: Well, many years. I don't know her exact background, but...

JOHNSON: Is she mistaken about what she saw?

LETBY: No.

JOHNSON: Dr Newby:

"I've put in inverted commas, 'Bruised areas on her abdomen, like evolving purpura'. There were two areas, they were sort of elongated areas. I couldn't give you the precise position on her abdomen, they were quite hard to describe in a way. It was almost like a sort of brown discolouration. They were sort of linear bruises. Bruises is the best way I can describe it, really."

That's 9 November, pages 20 to 22.

Is Dr Newby mistaken about what she saw?

LETBY: I can't confirm about the location of it, but yes, the description of it I agree with.

JOHNSON: She is not one of the Gang of Four?

LETBY: No.

JOHNSON: Dr Brunton:

"I've documented that the skin at this point was brown and there were areas of discolouration within that that were light brown, like 'evolving purpura across the trunk', it says. Because this is a completely unusual situation that I had never seen before, changes in the skin, I couldn't explain it."

That was 7 November last year, pages 35 to 37.

Is Dr Brunton mistaken about what he saw?

LETBY: No.

JOHNSON: On [Baby D]'s -- that was [Baby D]'s first collapse as far as he was concerned. On the second occasion she collapsed he said this:

"She had a bit of a rash, but again I can't remember the rash previously, but she had a little bit of a rash present on her abdomen."

Is he mistaken about that?

LETBY: I can't be specific at what time the different colouration was.

JOHNSON: The final time, just before [Baby D] died:

"I have observed that she had a bit of skin discolouration at that time, but I've documented -- I can't remember what it was like. I've never seen a baby behave in that manner prior to this and I've never seen a baby behave in that manner after this."

So a bit of skin discolouration at that time; is he mistaken about that?

LETBY: No.

JOHNSON: Finally, Dr Emily Thomas:

"I remember [Baby D] had an unusual episode where she came out in a rash. This occurred around 01.30 in the morning. I remember taking a look. I would describe the rash as almost having the appearance of a meningococcal-type rash. It was purple in colour and mainly over the abdomen. [Baby D] seemed to settle down and the rash appeared to fade."

That was an agreed statement and was read to the jury on 8 November. So although it was an agreed statement, out of an abundance of caution, I will ask you, do you dispute that?

LETBY: No, I agreed it looked purple and I agreed it looked like a meningococcal rash.

JOHNSON: And do you still not remember [Baby D]?

LETBY: I didn't recall [Baby D] at the time of my police interview, no.

JOHNSON: Do you remember her now?

LETBY: Yes.

JOHNSON: Do you remember the circumstances surrounding her death?

LETBY: No.

JOHNSON: Let's see what you were doing in the immediate aftermath of her death. If we go to tile 286, please. I'd be grateful if you'd read out your words, please, as they are your words.

LETBY: "We had such a rubbish night. Our job is just far too sad sometimes."

JOHNSON: And your friend [Nurse A] replied:

"No. What happened?"

And you replied?

LETBY: "We lost [Baby D]."

JOHNSON: And then [Nurse A]:

"What! But she was improving. What happened?"

And then:

"Wanna chat? I can't believe you were on again. You're having such a tough time."

Then you.

LETBY: "Messed about a couple of times and came out in this weird rash looking like overwhelming sepsis. Liz came in, re-screened, et cetera."

JOHNSON: So this is -- well, just go back to the one before, please. So this is a reference, isn't it, by you to "multiple collapses, messed about a couple of times"?

LETBY: Yes.

JOHNSON: "Came out in this weird rash"?

LETBY: Yes, I recall she had like a meningococcal rash, yes.

JOHNSON: "Looking like overwhelming sepsis"? Had you seen lots of meningococcal rashes?

LETBY: Not a lot, no.

JOHNSON: How many?

LETBY: I couldn't put a figure on it. I've seen it during my training on the children's ward.

JOHNSON: So several years earlier?

LETBY: Yes. And the discussion at the time was that the rash was due to overwhelming sepsis.

JOHNSON: "Liz came in."

That's Dr Newby, isn't it?

LETBY: Yes.

JOHNSON: "Re-screened, et cetera."

You did remember [Baby D] when you were interviewed by the police, didn't you?

LETBY: I remembered parts. I think it's -- I don't...

JOHNSON: Well, let's look at the next text and see whether that persuades you.

LETBY: "... then collapsed and had full resus. So upsetting for everyone. Parents absolutely distraught, dad screaming."

JOHNSON: "Parents absolutely distraught, dad screaming."

When you told the police you didn't really remember [Baby D], that was a lie, wasn't it?

LETBY: No, I didn't have any great recollection of the events, no.

JOHNSON: Tile 294, please.

LETBY: "Andrew and Liz said it'll probably be investigated."

JOHNSON: That's Drs Brunton and Newby; is that right?

LETBY: Yes.

JOHNSON: [Nurse A] replies:

"Dad was very anxious all day."

"[Query] what the delay in treatment".

And you replied, 297?

LETBY: "Just overall looking into the case."

JOHNSON: And at 298?

LETBY: "And reviewing what antibiotics she was on, et cetera, if it is sepsis."

JOHNSON: Then at 299?

LETBY: "Dad is beside himself. They only saw her briefly but don't want her to be moved out of 1. Didn't like family room but obviously can't really stay in 1 and got a 33-weeker about to deliver and we got two screens that have stayed."

JOHNSON: So two references now to [Father of Baby D]; yes?

LETBY: Yes.

JOHNSON: This was a dramatic and shocking incident, wasn't it?

LETBY: Yes.

JOHNSON: And you remembered it very well when you spoke to the police in reality, didn't you?

LETBY: No.

JOHNSON: Can we go to tile 307, please. Would you read this out?

LETBY: "Hmm, well, it's happened and that's it. Got to carry on. I finish for nearly 2 weeks after next Friday, so I'll keep ploughing on."

JOHNSON: Do you remember almost the first question I asked you about [Baby D]?

LETBY: Yes, was I affected.

JOHNSON: Yes. And I then said:

"Did you think: well, it's all happened and that's it?"

And your answer was?

LETBY: "Well, it's happened and that's it."

JOHNSON: No, you said that wasn't what you were thinking, was your answer. That was a lie, wasn't it?

LETBY: I don't think it's meant in the context that you're suggesting.

JOHNSON: Oh, well, you give us the context.

LETBY: That it's happened and we need to move forward and keep going. I don't think it's meant to be any insensitivity to the parents or [Baby D].

JOHNSON: You were enjoying all this, weren't you, Lucy Letby?

LETBY: No.

MR JOHNSON: My Lord, that's been almost exactly an hour.

MR JUSTICE GOSS: Yes. We'll have a break there then.

Thank you. 15 minutes, please.

(11.33 am)

(A short break)

(11.50 am)

MR JOHNSON: Something I forgot to ask you earlier but I'll ask you now: is there anything that you said yesterday or indeed have said this morning that you have reflected on and wish to reconsider?

LETBY: No.

JOHNSON: I have just been told that I've got to keep my voice up because this is being beamed to other courtrooms, but if you find the volume of my voice in any way intimidating, will you please say so? Will you say so?

LETBY: Yes.

JOHNSON: I'm not going to alter the content of the questions, just the volume at which they are delivered.

If we go back to the sequence of events, please, tile 360, please. I haven't asked you at all yet about your repeated searching of these parents on Facebook, okay, and I intend to ask you about them all together in one go towards the end. But in this particular case, given what you told the police about not remembering [Baby D], how did you remember the name of her mother, [Mother of Baby D], on 25 June?

LETBY: On 25 June I did recall [Baby D], in 2015.

JOHNSON: How did you recall the name of her parents?

LETBY: I don't know.

JOHNSON: Have you got a good memory for names?

LETBY: Yes.

JOHNSON: Do you have to keep a written record of the names?

LETBY: No.

JOHNSON: You carry them in your head; is that right?

LETBY: Yes.

JOHNSON: So when we, for example, see you in other cases searching many months or even longer, later, you still remember the names?

LETBY: Yes.

JOHNSON: Would you say you've got a good memory?

LETBY: Yes.

JOHNSON: Can we move on to 362, please. This was a message you sent to your friend, Minna. Would you read it, please?

LETBY: "Yes. Work has been awful."

JOHNSON: She then replied on the next tile:

"Oh dear. Staffing probe? [as read]"

And then you said?

LETBY: "We've had three unexpected deaths, transfer out. Few sick ones. Unit full."

JOHNSON: And then at 365?

LETBY: "What I've seen has really hit me tonight."

JOHNSON: And then she replied:

"Have you worked today?"

And you replied?

LETBY: "No, been off since Wednesday morning and now it's all hit me."

JOHNSON: She then replied:

"With all the Walton stuff and work stuff you..."

And then it morphs into the next one, it's a continuation:

"... could do with talking to a proper counsellor."

Then the next one she sent you:

"If you don't sleep well, et cetera, it will damage your health in the long run."

And you replied?

LETBY: "I just don't think I can."

JOHNSON: She then said:

"Why not?"

LETBY: "I can't talk about it now, I can't stop crying. I just need to get it out of my system."

JOHNSON: She replied:

"Okay. You know yourself what is best for you at present. Just think carefully whether you should be working if you feel so bad."

Followed by:

"Maybe you need to take time off."

And the conversation concluded with your next one, please.

LETBY: "Work is always my priority. I won't let it affect it. I just haven't let myself cry over it until now and been thinking about Walton, et cetera. Once I've let it out tonight, my head will be clearer."

JOHNSON: Yes. I think you've already told us, but in case anybody forgot, the reference to Walton was to an investigation you had at the Walton Centre in Liverpool; is that right?

LETBY: Yes.

JOHNSON: This was a very memorable time of your life, wasn't it?

Do you agree?

LETBY: Yes.

JOHNSON: The conversation with [Nurse A] then, please, a few days later on the following Tuesday, the 30th. If we could go to 382, please.

So she to you:

"Yeah. There's something odd about that night and the other three that went so suddenly."

And you replied?

LETBY: "What do you mean?"

JOHNSON: Did you really not understand what she meant?

LETBY: That's what I've replied at that time.

JOHNSON: I know. What I'm asking you is whether you really didn't understand what she meant.

(Pause)

Did you not understand what she meant?

LETBY: No.

JOHNSON: Or were you worried that people were starting to put two and two together?

LETBY: No.

JOHNSON: Well, we'll see from the next text you sent, following up, 2 minutes later:

"Odd that we lost 3 and in different circumstances?"

So this is a reference, isn't it, to [Baby A], [Baby C] and [Baby D]?

LETBY: Yes.

JOHNSON: All of whom had, on the evidence, odd-coloured or type of rashes that experienced medical professionals had never seen before? That's the evidence, isn't it?

LETBY: Yes.

JOHNSON: These were not different circumstances, were they?

LETBY: Yes, they were.

JOHNSON: Well, let's see what your friend thought, who had witnessed two of these incidents, the second being [Baby B], of course. 385:

"I dunno. Were they that different?"

Then:

"Ignore me. I'm speculating."

And you replied what, please?

LETBY: "Well [Baby C] was tiny, obviously compromised in utero. [Baby D] septic. It's [Baby A] I can't get my head around."

JOHNSON: Then [Nurse A]:

"Was she [in other words [Baby D]] definitely septic? Did the PM [post-mortem examination] confirm?"

And you?

LETBY: "I don't think the full PM is back yet. Debrief is next week but I'm away."

JOHNSON: Then she to you at 22.07, tile 390:

"When's [Baby A]'s? They were talking of doing a joint one for all three as all close together and similar in being full arrests in babies that were essentially stable. Dunno if they are doing though."

And your response, please?

LETBY: "Ah, not sure, but [Baby C]'s is Thursday and [Baby D] next week."

JOHNSON: Followed by?

LETBY: "No mention of [Baby A]."

JOHNSON: And then she said:

"I'd like to go to [Baby D] and [Baby A]'s."

And you?

LETBY: "Speak to Eirian. I think [Baby D]'s is the 6th."

JOHNSON: Then on the next tile, please, 395. We've moved on in time, more than 3 months, haven't we, from the death of [Baby D] by the time you were searching for [Father of Baby D], the distraught father?

LETBY: Yes.

JOHNSON: A name you were carrying in your head?

LETBY: Yes.

JOHNSON: The next tile, please. Another search a minute later for [Father of Baby D]. You didn't really forget [Baby D], did you, Lucy Letby?

LETBY: Yes, I didn't recall specific details at the time of my interview.

JOHNSON: Just before we leave the subject of Facebook, although we will return, does Facebook store the names that you search?

LETBY: Yes.

JOHNSON: How many names does it store?

LETBY: I don't know.

JOHNSON: Can I suggest the last six?

LETBY: I don't know. I can't answer that.

JOHNSON: So what I'm suggesting to you is that if you're on Facebook all the time, which you are, and when you go back days, weeks, months later, you have the source of the name either in your head or written down somewhere.

Are you saying in your case that it was in your head?

LETBY: Yes.

JOHNSON: Yes. And yet you say you couldn't remember [Baby D] when the police were asking you questions in interview?

LETBY: Yes, not specific details.

JOHNSON: Is it your case that staffing levels contributed to [Baby D]'s death?

LETBY: No.

JOHNSON: Is it your case that medical competence contributed?

LETBY: Yes, I believe she didn't have the appropriate treatment at the start of her life.

JOHNSON: So the delay with antibiotics?

LETBY: Yes. It may have had an impact on her, yes.

JOHNSON: They don't guard against air embolus, though, do they, antibiotics?

LETBY: No.

JOHNSON: Do you accept that once [Baby D] got into the neonatal unit she was treated properly?

LETBY: I haven't gone through all of her notes to able to make that assessment myself.

JOHNSON: Do you accept that by the end of the day shift, on 21 June, [Baby D] was stable with a minimal amount of respiratory support in no additional oxygen, she was responsive and was making good progress?

LETBY: I'd have to refer back to the notes. But yes, if that's what's documented.

JOHNSON: That's what Dr Rylance told us on 8 November from Switzerland.

LETBY: Okay.

JOHNSON: And it's what she wrote in her notes at tile 158.

Do you accept that Caroline Oakley's nursing notes are accurate?
Do you want to look at them?

LETBY: Are you asking me about a specific note or in general?

JOHNSON: To be fair to you, let's have a look. It's tile 207.

If ever you want to look at the evidence, you just let me know.

We see this is a note made after [Baby D]'s death. She recounts taking handover, doing the necessary checks, the position of lines. The fact that she was on, it says, NCAP in air.

Do you accept her notes?

LETBY: Yes.

JOHNSON: Do you remember her saying that [Baby D] was breathing beautifully in air and her blood saturations of oxygen are the highest they can be?

LETBY: I don't recall her exact turn of phrase, no.

JOHNSON: Let's look at T169, which is her observations. If we can just scroll down, please. Thank you. We see the respirations. Obviously things go wrong at around midnight or 1 o'clock in the morning.

If we look down, please, the temperature is pretty much on the money, isn't it?

LETBY: Yes.

JOHNSON: Respirations, fine?

LETBY: Yes.

JOHNSON: Anything there of concern?

LETBY: No.

JOHNSON: Do you accept the evidence of Caroline Oakley that she was on a break when [Baby D] collapsed?

LETBY: I don't recall. I don't know how to answer this sort of question.

JOHNSON: Well, she told the jury that she was on a break when [Baby D] collapsed. Do you accept that evidence or do you say that she wasn't on a break or something else?

LETBY: I couldn't say either way because I don't know.

JOHNSON: Well, you were there. You heard her evidence. Do you want to make any further comment about it?

LETBY: No.

JOHNSON: Let's go back to T168, please. If she was on a break who was the other nurse in the nursery?

LETBY: Myself.

JOHNSON: Do you remember Kathryn Percival-Ward or Percival-Calderbank confirming to the court that Caroline Oakley was on a break?

LETBY: Yes.

JOHNSON: So two of your colleagues, one of whom is a friend.

Do you accept that she was on a break?

LETBY: Yes.

JOHNSON: Can we look at the neonatal review, please, for [Baby D]?

I'd like to go to the page that's in the bottom right-hand corner, page 3 of 6. It's actually page 4 of 7 because you will remember, ladies and gentlemen, that we got an extra chart that was slotted in because the population distribution changed during the shift.

Do you see there at line 119, marked with a black line, the initial collapse of [Baby D]?

LETBY: Yes.

JOHNSON: Do we see that in the 30 minutes prior to [Baby D]'s collapse you are not recorded as doing anything for any of the children in the neonatal unit?

LETBY: Can you repeat that, please?

JOHNSON: Certainly. It might be best to concentrate on lines 109 down to 118. What I asked you was whether there is any record of you doing anything else for any of the other children in the neonatal unit in those 30 minutes or so.

LETBY: No, the last record is at 01.00.

JOHNSON: Yes. So that's line 108, which is the child MRE; is that right?

LETBY: Yes.

JOHNSON: Then there are two previous entries a quarter of an hour earlier at 00.45, lines 101 and 102?

LETBY: Yes.

JOHNSON: Then before that, at 15 minutes for JE; is that right?

LETBY: Yes.

JOHNSON: Can we go to tile 201, please, keeping that neonatal review open, please? Scroll down. Thank you.

The blood gas for [Baby D] taken at 01.14 was done by you, wasn't it?

LETBY: I don't know.

JOHNSON: That's your writing, isn't it?

LETBY: It could be my writing, yes.

JOHNSON: Yes, it is. You see where it says 01 and then the 14 in slightly elevated --

LETBY: Yes.

JOHNSON: -- numbers? That's the way you write these numbers.

LETBY: Yes.

JOHNSON: You're not the only person, of course, but that is the way you do it.

LETBY: Yes.

JOHNSON: And that is your writing, isn't it?

LETBY: It looks like my writing, yes.

JOHNSON: Yes. Why didn't you sign it?

LETBY: That's just an oversight, the same as the gas on the next line down hasn't been signed, it's just an error that happens from time to time.

JOHNSON: Yes, well, of course by the time of the subsequent gas, [Baby D] was in a pretty poor state, wasn't she, 02.22; is that right?

LETBY: The gas is good, I'm not sure about her clinical picture at that time. It's a good blood gas.

JOHNSON: Can we go to tile 202? You see there's some observations at 01.15? This line.

LETBY: Yes.

JOHNSON: If we scroll down, please. There's quite a lot of detail filled in there and it's signed by Caroline Oakley, isn't it?

LETBY: Yes.

JOHNSON: Do you remember she told the jury she got those details from "the girls" --

LETBY: No, I don't remember that.

JOHNSON: -- because she was on a break? You don't remember that bit of evidence?

LETBY: No.

JOHNSON: Were you looking after [Baby D] while she was on a break?

LETBY: No.

JOHNSON: Who was?

LETBY: I don't recall that there was -- who was allocated to look after her. I think Caroline had only gone on her break potentially 5 minutes before.

JOHNSON: Tile 203, please. You see the infusion chart?

LETBY: Yes.

JOHNSON: We have the infusion at 01.25, which is the same as what we see in the neonatal review at line 116.

LETBY: Yes.

JOHNSON: If anybody wants to make a note, it's the same as tile 203 in the sequence.

This is whilst Caroline Oakley was on her break, she said.

LETBY: I don't agree with that. When we sign medications, we sign at the time it is given.

JOHNSON: But it depends whether she's telling the truth about that, I suppose, doesn't it?

LETBY: Yes.

JOHNSON: Yes. Whose handwriting is the date and time in the date and time started column?

LETBY: It looks like mine.

JOHNSON: Did you take the opportunity whilst Caroline was absent to sabotage [Baby D]?

LETBY: No.

JOHNSON: When the alarms went off for [Baby D] you were standing over her though, weren't you?

LETBY: I don't recall.

JOHNSON: Well, who else could it have been? Go back to the population distribution chart, please, at tile 168.

Let's look at the candidates for who it could have been looking after [Baby D] in nursery 1. Caroline Oakley wasn't there. [Nurse C]. Is she a possibility?

LETBY: Yes.

JOHNSON: But she said that she wasn't there, so that depends on whether she's telling the truth or not.

MR MYERS: Or if her memory is accurate. We keep coming back to this business constantly about, oh, it's a test of who's telling the truth. It is a comment. We keep receiving it (inaudible) what the witness recalls she recalls. It doesn't require they must be lying because there are other issues that could be at play as to how accurate somebody is.

MR JUSTICE GOSS: Yes, if she is truthful and accurate.

MR MYERS: If her memory is accurate, yes.

MR JUSTICE GOSS: Exactly, if her memory is accurate.

MR JOHNSON: Who are the other possibilities?

Kathryn Percival-Ward?

LETBY: Yes.

JOHNSON: Who had, what, four children to be looked after?

LETBY: Yes.

JOHNSON: What about Liz Marshall, would she be in nursery 1?

LETBY: No.

JOHNSON: No. It was you, wasn't it?

LETBY: I can't confirm that. I will have been in nursery 1 with my babies, but other staff may have been in and out of the nursery at that time as well.

JOHNSON: Can we go to tile 221, please? There's an entry at 01.30. Do you see that?

LETBY: Yes.

JOHNSON: Whose handwriting is, "Oral secretions ++"?

LETBY: I can't say definitively.

JOHNSON: Whose does it look like?

LETBY: It could be mine.

JOHNSON: Yes. Why were you writing in [Baby D]'s chart, if it was you at 01.30, if you were not the person looking after her?

LETBY: It may have been something that I documented alongside Caroline Oakley, who's filled in the other columns.

JOHNSON: Yes, but if she's on her break, she can't have filled them in at the time, can she, if she's on her break?

LETBY: So you mean she's filled them in at 1.30, when she was on her break, retrospectively?

JOHNSON: Well, it all depends, doesn't it? If we move across, please, in the chart towards the right of it, do you see where it says, "Suction pp LL"?

LETBY: Yes.

JOHNSON: Whose writing is that?

LETBY: It's not mine. I would assume it's Caroline Oakley.

JOHNSON: Who's LL in this context?

LETBY: It would be me.

JOHNSON: Because I'm suggesting you were, to use Dr Jayaram's phrase, babysitting [Baby D]. You had, as a matter of fact, been in nursery 1 throughout, hadn't you?

LETBY: I can't -- I can't comment on that. I don't know for certain.

JOHNSON: Let's look at back at the neonatal review, please, if we can, starting at 120 in the neonatal review, line 120.

This is after [Baby D]'s first collapse now. We see that you make a note at 01.30. Is that right?

Do you see that?

LETBY: Um... Yes. For JE, yes.

JOHNSON: Yes, for JE. Were you really -- sorry, 120 is [Baby D] at 01.30.

LETBY: Yes, that's Caroline Oakley referring to me in her nursing notes, I believe.

JOHNSON: Oh, I see. Also at 01.30, line 122, you are, as you just told us, making an entry for JE?

LETBY: Yes.

JOHNSON: And we see, don't we, that you were putting something on to JE's, if the record is correct, neonatal fluid balance chart?

LETBY: Yes.

JOHNSON: Were you really doing that as [Baby D] was collapsing?

LETBY: I can't give a definitive time.

JOHNSON: Well, have you tried to make the paperwork look like you were doing something else at the time [Baby D] collapsed?

LETBY: No, I've not falsified any paperwork.

JOHNSON: Ever?

LETBY: No.

JOHNSON: Well, we'll come to that.

We see at 126, over the page, 127, 129 and 132, were these really being put in just after [Baby D] had collapsed?

LETBY: Medications, yes, they are signed for at a specific time, yes.

JOHNSON: 139 (inaudible)?

LETBY: Fluid balance charts and observations charts may not be definitively accurate of that time, no, but medication would be.

JOHNSON: At 155 to 162, we see that you were involved in giving medications to [Baby D] before her second collapse at about 3 am.

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: No.

JOHNSON: [Baby D]'s third collapse and final collapse we see at line 185, don't we?

LETBY: Yes.

JOHNSON: At 174, just before [Baby D] -- well, it's about -- I say just before, it's about 25 minutes before [Baby D]'s final collapse, you were setting up an infusion with Caroline Oakley for [Baby D], weren't you?

LETBY: Yes.

JOHNSON: We see that information if we could, please, Mr Murphy, at tile 237.

Despite all that, you're saying you don't or you didn't, when the police were asking you about it, really remember [Baby D]?

LETBY: That's right.

JOHNSON: [Baby D] died because you injected her with air, didn't you?

LETBY: No.

JOHNSON: And I'm suggesting that's the reason she had air in her great vessels, like [Baby A] and [Baby O].

LETBY: I did not give her air.

JOHNSON: Did you think that the paperwork wouldn't tie you to [Baby D] at the time she collapsed?

LETBY: Can you rephrase that, please?

JOHNSON: Yes. When you saw the paperwork when the police were asking you questions, did you think that there wasn't really a link between you and [Baby D] at the time of her collapse?

LETBY: No, I had other babies, I wasn't caring just for [Baby D].

JOHNSON: Did you deliberately take the decision just to say, "Well, I don't remember her", to avoid answering questions --

LETBY: No.

JOHNSON: -- or to try to avoid answering questions?

LETBY: No, I tried to be as cooperative as I could be.

JOHNSON: You did remember [Baby D], didn't you?

LETBY: Not in specific details at the time of the interview, no.

JOHNSON: Despite all that?

LETBY: Yes.

JOHNSON: Despite the fact that, as you have told us, you have a very good memory, particularly a good memory for names?

LETBY: Yes.

MR JOHNSON: My Lord, I'm going to move on to [Baby E].

MR JUSTICE GOSS: Yes.

LETBY: Can I have a break, please?

MR JOHNSON: I'm sure his Lordship will give you a break.

MR JUSTICE GOSS: Yes, certainly.

MR JOHNSON: Could I just tidy up one thing, which will take about 2 minutes?

MR JUSTICE GOSS: Certainly, yes.

MR JOHNSON: Thank you.

I'd like to give you a document, please.

(Handed)

This relates back to [Baby C]. It will only take 2 minutes. If Mr Murphy would assist by putting up J2038. His Lordship asked you a question about taking hand and footprints from a child.

LETBY: Yes.

JOHNSON: And he asked you specifically about whether that was done before or after death.

LETBY: Yes, and it can be done either way.

JOHNSON: I'm going to suggest to you that that is untrue.

You are lying about that, that it can be done before death in these circumstances.

LETBY: No, I don't agree.

JOHNSON: Well, let's look at the pro forma.

LETBY: We did...

JOHNSON: Sorry?

MR JUSTICE GOSS: "We did" -- do you want to say something?

LETBY: We do take footprints on babies that are alive, for example [Baby B] was alive and had footprints taken to go with [Baby A].

MR JOHNSON: Yes, but that was in very different circumstances, wasn't it? You don't take them of a child that is about to die.

LETBY: Yes, you can, yes.

JOHNSON: Well, not according to the checklist. Do you follow the rules or not?

LETBY: Yes.

JOHNSON: Right. What's the heading of this particular checklist that relates to what is done for children -- done for parents following the death of a child?

LETBY: Following neonatal death?

JOHNSON: Yes. There's nothing about "as they are dying" here, is there?

LETBY: It's very unusual to have a baby like [Baby C] that was dying for a prolonged period of time.

JOHNSON: And are you saying that it would be appropriate in those circumstances to intrude into those last moments with a parent to take hand and footprints?

LETBY: No, it may be something that they actually want to become involved with.

JOHNSON: Yes, but they didn't, did they?

LETBY: I don't -- I can't say that. I don't know.

JOHNSON: Is this your something writing on the form, as a matter of interest?

LETBY: No.

JOHNSON: Because actually, you shouldn't have been having anything to do with [Baby C] at this point, should you?

Should you?

LETBY: No.

JOHNSON: No.

MR JUSTICE GOSS: Right. 15-minute break? Right?

LETBY: Yes.

MR JUSTICE GOSS: Is that what you're wanting?

MR JOHNSON: Or perhaps take an early lunch.

MR JUSTICE GOSS: Why don't we take an early lunch rather than the inconvenience of coming back for 10 minutes or so. We'll start again at 1.45. We won't have a longer session time wise than we conventionally have. In other words, we won't go all the way to 4.15 because that's too long a period. We just push

everything forward a bit, which means we'll finish a bit earlier today.

You'll understand why I'm doing what I'm doing and why I am doing it.

Could you be ready to come in at between 1.45 and 1.50? Thank you.

(In the absence of the jury)

MR MYERS: My Lord, we're observing Ms Letby and we're grateful for the opportunity for her to have an early lunch. We are going to ask if Ms Santolini can see her over lunchtime.

MR JUSTICE GOSS: I agree. That had crossed my mind.

MR MYERS: We'll inform the court of anything that arises out of that.

MR JUSTICE GOSS: Yes, I was going to raise that in any event.

MR MYERS: Thank you.

MR JUSTICE GOSS: All right, thank you.

(12.34 pm)

(The short adjournment)

(1.45 pm)

(Delay in proceedings)

(2.47 pm)

MR JUSTICE GOSS: Mr Myers, I have received information that has led me to the conclusion that it would not be appropriate to continue this afternoon. We are about to embark upon a significant break again in the trial for reasons that are -- well, because it's been known for some time, so we will not be sitting again until Wednesday of next week. So I'm simply going to say to the jury that they should return on Wednesday at 10.30.

MR MYERS: We're grateful, my Lord. There's no further matter or request we raise at this stage.

MR JUSTICE GOSS: Thank you very much indeed.

(In the presence of the jury)

MR JUSTICE GOSS: Members of the jury, I sincerely apologise for the fact that you've been kept waiting for an hour this afternoon. In fact, we are not going to proceed any further for reasons with which you needn't concern yourselves. I have made the decision that we shouldn't continue this afternoon, so I'm going to bring today's hearing to an end.

As you know, it's not just the weekend that is coming up, it is then a two-day weekday break next week and our next sitting day will be Wednesday of next week at 10.30, when we will resume.

I'm sorry about all the inconvenience that's been caused. Could I just remind you and emphasise to you that you are trying this case on the evidence that's been placed before you. You are not to conduct any research about anything or anyone to do with this case and you are not to communicate with anyone in any way about anything or anyone to do with this case. Keep your thoughts to yourselves, you've had opportunity to sit and talk about it together, whether you've taken those or not, I don't know, but you are bound to be forming views and will have been for many weeks now, I anticipate, about aspects of the evidence. Just keep those thoughts to yourselves. All right? I'll see you at 10.30 on Wednesday morning.

Thank you very much indeed.

(In the absence of the jury)

MR JUSTICE GOSS: In relation to those representatives of the media who are present in court, I don't know whether you individually are aware, but I've received some representations in relation to another media organisation being able to have a place in this courtroom.

Mr Worthington, the clerk of the court, is going to address that matter. He will acquaint you with what the situation is and discuss it. I'm sure it can be resolved. It's not of huge significance, but I understand what is requested and why. All right?

Thank you very much.

Nothing untoward so far as you're concerned, but it may just have repercussions in relation to who the particular individuals are in here. Nothing else then, Mr Myers, Mr Johnson?

MR MYERS: No, thank you.

MR JOHNSON: No --

MR MYERS: We're grateful to the court for the time taken to deal with matters this afternoon.

MR JUSTICE GOSS: Ms Letby, you understand that is the end of the proceedings for today. You are still in the same position as you have been up to today. You cannot communicate with your legal team, you understand that, and the next day you will be brought to court will be Wednesday of next week. Thank you.

For the assistance of you, Mr Myers, and you, Mr Johnson, I will be in this building on Tuesday of next week.

MR MYERS: Thank you, my Lord.

MR JOHNSON: Thank you.

MR JUSTICE GOSS: So I'm not going on holiday or anything like that.

MR MYERS: We're grateful.

(2.53 pm)

(The court adjourned until 10.30 am on Wednesday, 24 May 2023)